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27th march, 11:59pm

Critical Review: Ruby's Encounter in Primary Health Care

The case of Ruby, an Aboriginal woman, and Sue, a non-Indigenous health worker, shows how deeply flawed the primary health care delivery system is. This critical analysis examines the situation whereby Sue took away the family support system of Ruby, sat down in a hasty consultation behind a desk and handed her impersonal pamphlets-which she threw in the waste bin. One of the most prominent problems that were manifested in this interaction is the rejection of Aboriginal kinship systems, the establishment of a biomedical power, and the lack of culturally safe practice altogether. Based on the Social and Emotional Wellbeing (SEWB) framework as described by Gee et al. (2014), and critical analyses of whiteness and decolonising practice, this review holds the view that the approach provided by Sue is an example of colonised health care which puts institutional efficiency over relational care. This analysis will discuss how this interaction was framed by privilege and whiteness, use SEWB principles to the consultation, and decolonising and culturally safe alternatives that prioritize Aboriginal ways of knowing, being, and doing.

The modern Australian health system exists in a perpetuated colonial framework that has been persistently contributing to unfair health outcomes among Aboriginal and Torres Strait Islander Peoples. According to Gee et al. (2014, p. 56), the National Aboriginal Health Strategy Working Party recorded that the dominant society designs and controls mental health services, resulting in one of the biggest discrepancies between the service provider and user- the dominant society. This is not simply a gap but a structural one- it is the inability of mainstream services to acknowledge or to accommodate programs to the Aboriginal beliefs, laws and ways of knowing. An example of this is the case of Ruby consultation. The room layout of the consultation, where Sue is in the back of a desk and Ruby is on the opposite side, replicates the hierarchical power structures which situate the health professional as an expert and the aboriginal client as a passive recipient (Browne et al., 2016).

According to Mackean and Freeman (2025), the mainstream health care continues to perpetuate the harms of ongoing colonisation, which include racism, lack of access and quality of care, and undermining Aboriginal and Torres Strait Islander knowledges and conceptions of health. The fact that Sue does not include the mother, aunty and grandmother of the patient in the consultation room is another clear indication of how colonial health systems put a special emphasis on individualised, nuclear-family frameworks instead of Aboriginal kinship frameworks, which have been a key component of social organisation and wellbeing throughout tens of thousands of years (Dudgeon et al., 2017). This is crucial to the historical context. Colonisation systematically destroyed the connection of Aboriginal and Torres Strait Islander Peoples to land, culture, family and community, connection of which Gee et al. (2014, p. 59) define as key areas of SEWB. When health services remain a colonial prism, they do not merely help, but, in fact, reproduce the dispossession and disconnection that colonisation brought. Freeman (2025) also adds that unless

the governments, services and funders take positive action to turn those bottom-line realities around, they are not neutral- they are keeping the colonised practice alive.

The idea of whiteness is not just about the identity of the individual or colour of skin. According to Gatwiri, Rotumah, and Rix (2021), whiteness is an ideology and organising logic that serves to maintain settler colonial power, deny Indigenous sovereignty, and re-tell racial hierarchies. Within the health setting, whiteness functions as a norm that is not visible, white ways of knowing, being and doing is being framed as a neutral, professional, and superior and the Aboriginal and Torres Strait Islander Peoples are constructed as a cultural, racialised, and constantly watched location (Watego et al., 2025). The way that Sue treats Ruby is whitened in several different ways. To start with, there is invisibility and normativity of the approach of Sue- she does not analyze her own positionality and takes into consideration the possibility that her standard procedure could be culturally out of order. Gatwiri et al. (2021) describe the ways in which white views are regarded as common sense and are never questioned, and Indigenous people are formed as problematic or disobedient.

Second, the rush distribution of pamphlets and the question of do you understand by Sue is an example of pathologisation of Aboriginal bodies and the presumption of a lack. Watego et al. (2025) contend that whiteness in health systems diverts the focus to the individual power of race and cultural misperception or a lack of communication, which enables the continuation of racial violence without confronting it or stopping it. Sue fails to inquire about the situation of Ruby, her values and support requirements. By throwing away the pamphlets, Ruby is not denying health care but a type of care that is not relevant to her life and is out of touch (Bond et al., 2019). Third, the family has been excluded of Ruby, showing the effect of whiteness as an institutional force. Mackay, Lee-Tory, Smith, Molloy, and Clapham (2025) point to the concept of whiteness in nursing as an issue inherent to the Australian health professional practice- assumptions that have never been questioned regarding the appropriate care provision, which systematically erases the Aboriginal knowledge of care. According to Gee et al. (2014, p. 62), family and kinship systems position people in the community in relationships of caring, sharing, obligation and reciprocity. In breaking such ties, Sue unwittingly repeats the colonial effort of severing ties between Aboriginal people and their kin (Durey & Thompson, 2016).

Social and Emotional Wellbeing framework is a paradigm shift in health care. According to Gee et al. (2014, p. 55), SEWB is a multidimensional concept of health that incorporates mental health, but also includes connection to land, culture, spirituality, ancestry, family and community. In contrast to the biomedical paradigm, SEWB places health within an ecology of relationships, which is holistic to body, mind, emotions, family, community, culture, land and spirituality (Dudgeon et al., 2017). There are nine principles that guide it: health as holistic; self-determination; cultural understanding; impact of history in trauma and loss; human rights; impact of racism and stigma; centrality of kinship; cultural diversity; and recognition of Aboriginal strengths (Gee et al., 2014, p. 58). The consultation of Sue contravenes almost all the principles.

Consider self-determination. Ruby does not even get a question of what she wishes and who she wishes to be present. In the study by Webb, Meldrum, Wapau, Sagigi, Quigley, Strivens and Russell (2024), the authors discovered that the words used by First Nations peoples to refer to low SEWB were: worry, sad, and stress. A consultation which fails to provide space by which Ruby can air her concerns cannot handle her SEWB. The most evident principle that is violated is the centrality of kinship. According to Gee et al. (2014, p. 62), the kinship systems offered a highly secure attachment system that has developed caring relationships. Ruby clearly explains that she has the support of her mother, aunty and grandmother. By not including them, Sue denies the support system of Ruby and compromises the relational basis of her wellbeing (Browne et al., 2016). The concept of the identification of the influence of history, trauma, and loss is also important. The fact that Ruby comes in alone is not to be interpreted as passivity. Since the removal of Aboriginal children to families is a history, when health professionals were involved in such a decision, it is understandable that Ruby was reluctant to be in an institutional environment (Dudgeon and Walker, 2015). Health professionals are on the risk of retraumatising the very people they are supposed to help unless they are conscious of their past.

Decolonising practice involves not only the inclusion of cultural awareness training. According to Mackean and Freeman (2025), decolonising practice can be defined as ways of working that unpack colonial approaches to health, such that Aboriginal and Torres Strait Islander peoples can not just survive but also thrive. Most importantly, decolonising practice is directed by Aboriginal ways of knowing, being and doing, dismantles systemic racism, confronts power imbalances, recognizes and responds to white privilege (Mackean and Freeman, 2025). How would decolonising practice look like to Ruby? First, Sue ought to have invited the family of Ruby in the consultation. Instead of thinking that it is the nuclear family model, Sue might have questioned: Who would you prefer to be with you? It would have recognized that Ruby had an agency and honored her kinship ties (Bond et al., 2019).

Second, Sue ought to have gotten off the desk. Seating next to Ruby freely would be a sign of cooperation rather than questioning her. Mackay et al. (2025) posit that decolonising nursing practice demands the need of a cultural safety framework in order to deal with historical injustices. The desk is neutral-a colonial object (Durey & Thompson, 2016). Third, Sue ought to have inquired with Ruby what she needed, as opposed to believing that pamphlets were the solution. Ruby is a mother of three young children with a six-week-old baby, who might require practical assistance, respite, or even an empathetic listener. The pamphlets in the bin indicate that Sue has not offered meaningful care, but not due to non-compliance by Ruby (Watego et al., 2025). Fourth, Sue ought to have focused on relationship over clinical content. The initial visit needs to focus on yarning- an Aboriginal relationship approach that establishes trust before dealing with particular health issues. To comprehend the ways in which SEWB is viewed by communities, Webb et al. (2024) employed yarning as an Australian First Nations relational approach. Freeman (2025) also

stresses that the basis of decolonising practice should be based on a robust Aboriginal and Torres Strait Islander workforce. Sue is ethically obliged to practice critical reflection, pursue cultural supervision, and proactively transform power relationships, among other non-Indigenous practitioners.

The empowerment of Aboriginal health contexts bears absolutely different meanings of self-efficacy as an individualistic concept. One of the principles that guide SEWB is the right to self-determination, which Gee et al. (2014, p. 57) claim is a clinical framework rather than an abstract concept. Empowerment implies that Aboriginal people should be in control of any decisions impacting their health (Dudgeon et al., 2017). In the case of Ruby, empowerment would involve Sue posing questions such as: What is the matter with you? instead of What matters to you? Ruby is aware of her life, the challenges of raising three young children, the pressure of her husband working FIFO, the importance of her family. She requires a health system that embraces her reality. The empowerment model would include having Sue request and collaborate with Ruby to get the support she desires, which could include flexible appointments, home visits, or linking her to Aboriginal family programs (Bond et al., 2019). The 2004 SEWB framework (cited in Gee et al., 2014, p. 68) describes that SEWB problems include grief, loss, trauma, abuse, violence, cultural dislocation, racism, and social disadvantage. Most of them are not medical in nature. Without empowering Aboriginal people to deal with these determinants, a health system would not be able to make sustainable gains in health (Durey & Thompson, 2016).

The consultation of Sue cannot be out of context. Health workers do not only have a role to play in personal interactions, they are not just placed in systems that reproduce colonial authority, they are also placed in structures that oppose it. Mackean and Freeman (2025) suggest that the decolonisation practice needs health professionals to participate in the pushback against abusive government policies and promote the flexible models of funding. To the pupil who witnesses this encounter, the pamphlets that are thrown away symbolize one of the most important lessons, not a one-time instance of ineffective communication but a systemic trend of culturally insecure care (Watego et al., 2025). It is the role of the student not to normalise this trend: raise their voice during supervision, push to change practices, and be able to change institutions (Dudgeon & Walker, 2015). Freeman (2025) remarks that those in power have long been aware that Aboriginal led practice is superior, yet systemic racism is difficult to change- that ingrained notion that white professionals are the best. Health professionals who are conscious of this have an ethical duty to name it, challenge it, and to take positive action to shift power to Aboriginal communities.

Through the lens of colonisation, whiteness, SEWB, decolonising practice and empowerment, this critical review has examined the interaction between Ruby and Sue. The colonised health care of Sue, separating Ruby and her family, sitting behind a desk giving impersonal pamphlets, is an example of the focus on efficiency and the lack of relational care. Privilege and whiteness worked within invisibility of the assumptions of Sue, the pathologisation of the position of Ruby, and the blaming on the non-compliance of Ruby. SEWB framework

focuses on kinship, self-determination, cultural understanding and historical awareness as the key to the successful primary health care. Decolonising practice would have invited the family of Ruby to see them and would have taken the family out of the back of the desk and would have inquired of Ruby what she required and would have put relationship first and duty second. Empowerment strategies acknowledge that Ruby needs to be in control of the decisions that impact on her health. The health professionals should become the instruments of social change- changing the systems which reproduce the colonial harm. To achieve ethical and effective primary health care, whiteness has to be dismantled and Aboriginal ways of knowing, being and doing should be centred.

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Gen AI declaration

I confirm that I, Kaushal Khanal, have not knowingly or intentionally used generative AI to complete any part of this assessment."